DPP	DEPARTMENTAL POLICY AND PROCEDURE		
Title	**NASOGASTRIC TUBE INSERTION**		
Department	**Neonatology Department**		
Applies to	**All NICU Medical and Nursing Staff**		
Policy Code and Version Number	**DPP-NICU-021 (v2)**		
Last Version Approval Date	**28/1/2022**	Number of Pages	**4**

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APPROVAL DATE	EFFECTIVE DATE	TYPE OF REVISION	REVIEW DATE
19 / 3 / 2024	7 / 4 / 2024	TRIENNIAL	18 / 3 / 2027

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1.0 PURPOSE

1.1. To decompress the stomach thus gastric distention, nausea and vomiting.

1.2. To administer tube feeding and medication to patient unable to eat by mouth or swallow a sufficient diet without aspirating food or fluids into the lungs.

1.3. To remove stomach contents for laboratory analysis.

1.4. To lavage the stomach in case of poisoning or overdose of medications

2.0 DEFINITION/ABBREVIATION

2.1. **Nasogastric tube is a tube** (rubber or plastic tube) with radiopaque marker or strip at the distal end passed into the stomach via the nose to remove gas or stomach contents or for decompression post operatively and for feeding purposes.

3.0 RESPONSIBILITY

3.1. NICU Physician

3.2. NICU Nurses

4.0 POLICY

4.1. Nasogastric tube may be inserted by the physician or by a qualified nurse with the order of the physician.

4.2. Insertion of nasogastric tube requires close observation of the patient and verification of proper placement by x-ray studies.

4.3. Nasogastric tube must be changed every seven days unless ordered by a physician.

4.4. The date of change must be recorded in the nursing care plan and nursing notes.

4.5. The length of the nasogastric tube must be measured from the tip of the nose to the bottom of the earlobe to the end of xiphoid process.

5.0 PROCEDURE (S)

5.1. Explain to the patient or parents of children about the procedure.

5.2. Position the patient to a high fowler's position if health permits and support the head on a pillow.

5.3. Prepare equipment at bedside:

5.3.1. Nasogastric tube # 5-10 French. If rubber tube is used, place it on ice

5.3.2. If a plastic tube is being used, place it in warm water

5.3.3. Non-allergenic adhesive tape

5.3.4. Gloves

5.3.5. Water soluble lubricant

5.3.6. Stethoscope

5.3.7. Syringe as irrigation set

5.3.8. Suction equipment if required

5.3.9. Restraints

5.3.10. Gauze swabs

5.4. Determine how far to insert the tube. Measure from the tip of the nose to tip of the earlobe to the end of xiphoid process.

5.5. Wash hands and don gloves.

5.6. Lubricate the tip of the tube with water soluble lubricant.

5.7. Insert the tube, with its natural curve toward the patient into the selected nostril. Hyperextend the neck and gently advance the tube toward the nasopharynx.

5.8. Do not hyperextend or hyper flex an infant's neck:

5.8.1. If the patient swallows, passage of the catheter may be synchronized with the swallowing.

5.8.2. Do not push against resistance. Gently try rotating the tube if resistance is met

5.8.3. If there is no swallowing, insert the catheter smoothly and quickly

5.8.4. If infant especially, observe for vagal stimulations

5.9. Once catheter has been inserted to the pre measured length, ascertain correct tube placement by:

5.9.1. Inject 0.5-1 ml. air for small infants and up to 5 ml. in larger children into the catheter while simultaneously listening with stethoscope the typical gurgling or growling sound over the hypogastrium

5.9.2. Aspirate injected air from the stomach.

5.9.3. Aspirate small amount of stomach content and test acidity by pH tape.

5.10. Secure the tube by taping to the bridge of the patient's nose and bring split ends under the tubing's and back up over the nose.

5.11. For infants or small children, tape the tube to the area between the end of the nares and the upper lip as well as to the cheek.

5.12. Record the date & time the NGT inserted, type and size of the tube and length of the tube insert and document patient's response to the procedure

6.0 EQUIPMENTS

6.1. Nasogastric tube # 5-10 French, if rubber tube is used, place it on ice. If a plastic tube is being used, place it in warm water
6.2. Non-allergenic adhesive tape
6.3. Gloves
6.4. Water soluble lubricant
6.5. Stethoscope
6.6. Syringe as irrigation set
6.7. Suction equipment if required
6.8. Restraints
6.9. Gauze swab

7.0 ATTACHMENT/FORMS

7.1. Nurses Progress Notes (GDOH-NUR-NPN-203)
7.2. Physician Order Form (GDOH-INP-POF-079)
7.3. Laboratory Request

8.0 REFERENCES

8.1. MOH-KSA Guidelines for Neonatal Care

8.2. Photo Guide of Nursing Skills 2004, by Smith, Duell, Martin

8.3. Lippincott Manual of Nursing Practice, 7th Edition by Nettina

8.4. Fundamentals of Nursing 7th Edition, by Kozier, Erb, Berman, Snyder

8.5. CBAHI Standards NICU 3rd version

9.0 REVISION CLAUSE

9.1. Modification of policy format, lay out and policy coding

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Al-Mikhwah General Hospital

KINGDOM OF SAUDI ARABIA

وزارة الصحة
Ministry of Health

Hospital: _____ مستشفى:

Region: _____ المنطقة/المحافظة:

Dept./Unit: _____ القسم/الوحدة:

MRN: | | | | | | | | | | رقم الملف الطبي:

Name: _____ الاسم:

Nationality: _____ الجنسية:

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days العمر:

Date of Birth: ____ / ____ / 14____ H ____ / ____ / 20____ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

NURSES PROGRESS NOTES FORM

DATE &TIME	NURSES NOTES (D-data A-action R-response)	REGISTERED NURSE INITIAL WITH ID NUMBER

Note: Write the time in each entry & affix your initial at the end of each paragraph. Document your complete Name, Initial, Job number, Date & Time at the closure of your documentation. Draw a line across empty spaces.

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Name: _____ الاسم:

Nationality: _____ الجنسية:

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days العمر:

Date of Birth: ____ / ____ / 14____ H ____ / ____ / 20____ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

PHYSICIAN ORDER FORM

ALLERGIC TO:

DATE	TIME	PHYSICIAN ORDERS	PHYSICIAN STAMP / SIGNATURE	RN INITIAL / JOB NUMBER

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Nationality: _____ الجنسية:

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days العمر:

Date of Birth: ____ / ____ / 14____ H ____ / ____ / 20____ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

HAEMATOLOGY FORM نموذج فحص الدم

☐ Routine ☐ Urgent ☐ Pre-operative

Clinical Comments: .....

Requested by: ..... Stamp&Signature: ..... Date: ___/___/_____

Test	Result	Normal Range	Test	Result	Normal Range	Test	Result
CBC:			**DIFF. POLY**		**(40 – 75) %**	**RBC Morpho**	
WBCX 10^9		**(3.5 – 10.0)/L)**					
RBCX10^{12}		**(M 4-5 – 6.5 / F 40 - 5.0) / L**	**Band**			**BLD Parasites**	
Hb		**(M 13 – 18 / F 122 – 16) g/dL**	**Lymph**		**(20 - 45) %**		
Hct		**(M-40 – 54 / F-37 – 47) Ratio**	**Mono**		**(2 – 10) %**	**ESR: 1 hour**	**N(M 4-10 / F 6-12) mm**
Mcv		**(89-96) FL**	**EOS**		**(1 – 6) %**	**Sicking Test**	
Mc H		**(27-32) Pg**	**Baso**		**(0 – 1)**	**L.E. Prep:**	
Mc Hc		**(30-35) g/dL**				**Others:**	
Plix 10		**(150-400)**	**NRBC /100WBC**				
Retic		**(0.2-2) %**					

Remarks:

Pathologist: ..... Examiner: ..... Date: ____/ ____/_____

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